

# PAWSIC Center Self-Assessment

## Skin Health & Wound Management — Quality Improvement Audit Tool

### QUALITY IMPROVEMENT TOOL — PROTECTED & CONFIDENTIAL

This is a Quality Improvement (QI) tool prepared for and used exclusively by the facility's Quality Assessment and Assurance (QAA) / QAPI Committee in accordance with 42 CFR § 483.75(g) and (h). It is privileged, confidential, and protected from disclosure under federal and applicable state law, except for the limited purpose of demonstrating compliance with QAPI regulations.

**Purpose & Scope.** This comprehensive Skin Health and Wound Program Process Improvement Assessment provides a deep-dive evaluation of your center's clinical, operational, and structural health — spanning patient-centered care, team roles, interventions, process, and infection control. Completing it requires direct observation, medical-record review, and interviews with patients, leadership, and staff.

**When to use.** Complete annually (minimum), quarterly (optional), pre-survey, to support a plan of correction, or after a significant change such as an F686 or F880 citation, leadership turnover, or a change in the staffing model.

**How it is organized.** Part 1 surfaces high-level systemic risks that influence the success of clinical interventions and should be reflected on when crafting the final project plan. Part 2 scores the core constructs of a Center of Excellence across five sections. Scores can be tracked over time to analyze the trajectory of process improvement.

### How to Score — Keys & Legend

#### Likert Scale (0–5):

|   |                                        |   |                                                |   |                                            |
|---|----------------------------------------|---|------------------------------------------------|---|--------------------------------------------|
| 0 | Not in place                           | 2 | Implementation plan in place with a start date | 4 | Up and running with some improvement needs |
| 1 | Recognized as a need, but not in place | 3 | Implemented and in evaluation phase            | 5 | Running smoothly / Excellent               |

#### Audit Method (record in the Method column):

**O** = Observe • **I** = Interview (patient / staff) • **R** = Record / Document Review. Most criteria are validated by more than one method ("Ask, Observe, Validate").

#### F-Tag Quick Key (CMS SOM Appendix PP):

|             |                                                                   |             |                                                                     |
|-------------|-------------------------------------------------------------------|-------------|---------------------------------------------------------------------|
| <b>F686</b> | Treatment / services to prevent & heal pressure ulcers and wounds | <b>F880</b> | Infection prevention & control (incl. Enhanced Barrier Precautions) |
| <b>F684</b> | Quality of care — each resident receives necessary care/services  | <b>F881</b> | Antibiotic stewardship                                              |
| <b>F603</b> | Freedom from involuntary seclusion (avoid over-isolation)         | <b>F656</b> | Comprehensive person-centered care plan                             |

## Part 1 — F686 / F880 Foundational Risk Analysis (Staffing & Stability)

Scored separately to identify high-level systemic risks that impact the success of clinical interventions. Mark each as Met / Partially Met / Not Met and note evidence.

| Evaluation Criteria                                                                                                                                                                                    | F-Tag              | Status (Met / Partial / Not Met) & Comments |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|---------------------------------------------|
| <b>Survey History:</b> No F686 or F880 deficiencies in the last 2 years.                                                                                                                               | <b>F686 / F880</b> |                                             |
| <b>Leadership Stability:</b> DON and Administrator in place for the last 2 years.                                                                                                                      | <b>F686</b>        |                                             |
| <b>Nursing Turnover:</b> Licensed nursing staff turnover is < 20% over the last 2 years.                                                                                                               | <b>F686</b>        |                                             |
| <b>Agency Use:</b> Center utilized < 10% agency licensed staffing in the last 2 years.                                                                                                                 | <b>F686</b>        |                                             |
| <b>Root-Cause Discipline:</b> Any and all in-house acquired pressure injuries result in a team-based CEP root cause analysis that is completed, reviewed, and results in action.                       | <b>F686 / F684</b> |                                             |
| <b>CNA Consistency:</b> Stable CNA staffing with routine, set patient assignments.                                                                                                                     | <b>F684</b>        |                                             |
| <b>Clinical Outcomes:</b> No in-house acquired Stage 3, 4, or Unstageable pressure injuries over the last 3 months.                                                                                    | <b>F686</b>        |                                             |
| <b>Routine Prevention:</b> Application of lotions/creams to limbs is a routine ADL service.                                                                                                            | <b>F686 / F684</b> |                                             |
| <b>Transparency &amp; Imaging:</b> Reporting all skin issues is an expectation for all staff. Wound imaging is a standard process embedded in the EMR to ensure appropriate diagnosis and measurement. | <b>F686 / F656</b> |                                             |
| <b>Infection-Control Standing:</b> No F880 citation, or any prior F880 finding has a closed, verified corrective action plan; Enhanced Barrier Precautions are operational center-wide.                | <b>F880</b>        |                                             |

## Part 2 — Program Process Assessment

Score each criterion 0–5 using the Likert key. Enter the audit method(s) used and supporting evidence. Total each section, then carry totals to the Final Scoring page.

### Section 1 — Patient-Centered Care

| Evaluation Criteria                                                                                                                                                                                                                                                                  | F-Tag       | Method | Score (0–5) | Auditor Notes / Evidence |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|--------|-------------|--------------------------|
| <b>Dignified Skin Checks:</b> Permission requested, curtains pulled, patient covered, and explanation provided every time. Completed by 2 nurses, or nurse plus CNA/therapist, ON ADMISSION, weekly, and at change of condition.                                                     | F686 / F684 | O, I   |             |                          |
| <b>Risk Assessment:</b> Captures all risk factors and matches to interventions on admission/readmission, weekly x4, quarterly, and change in condition: moisture, mobility, friction/shear, nutrition, sensory/cognition, history of wounds, vascular impairment, dialysis, devices. | F686 / F656 | R, I   |             |                          |
| <b>Care Plan Integrity:</b> Focuses on impaired integrity / wound types and identifies risk factors that may impair healing, with matched interventions.                                                                                                                             | F656 / F686 | R      |             |                          |
| <b>Bedside Reality:</b> Patient/RP and CNA confirm listed interventions are occurring in real time; interventions are observed to be in place as intended.                                                                                                                           | F684 / F656 | O, I   |             |                          |
| <b>Inclusive Planning:</b> Changes to the care plan include communication with the patient/RP and CNA; education is a formal, documented intervention.                                                                                                                               | F656 / F684 | R, I   |             |                          |
| <b>Imaging-Guided Education:</b> Wound imaging is used for accurate diagnosis and to guide patient/family education, reflected in the plan of care.                                                                                                                                  | F686 / F656 | R, O   |             |                          |
| SECTION 1 TOTAL SCORE:                                                                                                                                                                                                                                                               |             |        | ____ / 30   |                          |

### Section 2 — Multidisciplinary Team Roles & Engagement

| Evaluation Criteria                                                                                                                                                                                                                                | F-Tag       | Method | Score (0–5) | Auditor Notes / Evidence |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|--------|-------------|--------------------------|
| <b>Medical Director Oversight:</b> Active in creating, approving, or supporting skin/wound protocols and in the selection and oversight of a Mobile Wound Group (MWG) or Certified Wound Consultant.                                               | F686 / F841 | R, I   |             |                          |
| <b>Mobile Wound Group / Certified Wound Expert:</b> Attends weekly rounds as contracted for agreed patient/wound types; provides imaging/diagnostics, recommendations, procedures, and documentation meeting F686, formulary, and facility policy. | F686        | R, O   |             |                          |
| <b>Team Leader Role:</b> A center-based Skin Health & Wound Management leader oversees, mentors, and coaches the program.                                                                                                                          | F686 / F684 | I, O   |             |                          |

| Evaluation Criteria                                                                                                                                                         | F-Tag       | Method | Score (0–5) | Auditor Notes / Evidence |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|--------|-------------|--------------------------|
| <b>Interdisciplinary Support:</b> RD (nutrition), SW (equity/discharge), and Therapy (mobility, modalities, positioning, debridement, swallowing, off-loading) are engaged. | F684 / F692 | R, I   |             |                          |
| <b>Admin / DON Support:</b> Oversight of operations, spend, pharmaceuticals, and Part B supply management.                                                                  | F684 / F865 | I, R   |             |                          |
| <b>Nursing / CNA Synergy:</b> Accurate RN patient/wound assessment, LPN wound treatment provision, and CNA intervention delivery are coordinated and reliable.              | F686 / F684 | O, I   |             |                          |
| SECTION 2 TOTAL SCORE:                                                                                                                                                      |             |        | ____ / 30   |                          |

### Section 3 — Interventions & Product Management

| Evaluation Criteria                                                                                                                                                                          | F-Tag              | Method | Score (0–5) | Auditor Notes / Evidence |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|--------|-------------|--------------------------|
| <b>Product Review Committee:</b> Formal process for evaluating and implementing product changes; formulary gaps and equivalents are defined.                                                 | F686 / F865        | R, I   |             |                          |
| <b>Utilization &amp; Spend Oversight:</b> Committee reviews reports for owned/rental products to prevent waste and “creep.”                                                                  | F865 / F684        | R      |             |                          |
| <b>Surface Safety:</b> Specialty beds/chairs evaluated for entrapment, restraint, safety, mobility, infection control, and healing.                                                          | F686 / F604 / F880 | O, R   |             |                          |
| <b>Heel &amp; Positioning:</b> Consistent use of heel off-loading and positioning wedges.                                                                                                    | F686               | O      |             |                          |
| <b>Supply Protocols:</b> Categories for basic care and debridement kits meet center protocols (cleanser, hydrogel, hydrofiber/alginate, hydrocolloid, foams, collagen, surfactant/debrider). | F686 / F880        | R, O   |             |                          |
| <b>NPWT Stewardship:</b> Appropriate use, tracking, and outcomes for Negative Pressure Wound Therapy.                                                                                        | F686               | R, O   |             |                          |
| SECTION 3 TOTAL SCORE:                                                                                                                                                                       |                    |        | ____ / 30   |                          |

### Section 4 — Process & Protocols

| Evaluation Criteria                                                                                                                                             | F-Tag       | Method | Score (0–5) | Auditor Notes / Evidence |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|--------|-------------|--------------------------|
| <b>Root Cause (CEP):</b> Team completes the CMS Critical Element Pathway for all in-house Stage 3, 4, Unstageable, and DTIs, and for declining/infected wounds. | F686 / F684 | R      |             |                          |
| <b>Wound Log &amp; MDS:</b> Real-time wound log matches MDS Section M; includes every skin impairment (POA noted); audited monthly against TAR and care plan.   | F686 / F641 | R      |             |                          |

| Evaluation Criteria                                                                                                                                                                                                  | F-Tag       | Method | Score (0–5) | Auditor Notes / Evidence |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------|--------|-------------|--------------------------|
| <b>Dignified Wound Rounds:</b> Weekly interdisciplinary rounds are respectful; patients are notified in advance and pre-medicated when applicable.                                                                   | F686 / F684 | O, I   |             |                          |
| <b>Communication Alerts &amp; Tracking:</b> Triggers for new wounds, declines, infection risk, or missed/refused treatments route to the provider and team same shift.                                               | F686 / F580 | R, I   |             |                          |
| <b>Annual Surface Audit:</b> Maintenance and clinical leads perform bed-surface viability and entrapment evaluations on a documented annual schedule.                                                                | F686 / F908 | R, O   |             |                          |
| <b>Monthly Sample Audit:</b> High-risk patients audited across TAR, Care Plan, Wound & Risk Assessment, Skin Check, and intervention checks (turning/positioning, off-loading, boots, cushions, surface, nutrition). | F686 / F684 | R, O   |             |                          |
| SECTION 4 TOTAL SCORE:                                                                                                                                                                                               |             |        | ____ / 30   |                          |

## Section 5 — Infection Control & Enhanced Barrier Precautions (F880)

*Infection-control requirements govern every dressing change — F880 (infection control) and F686 (wound care) are inseparable. F880 is the most-cited deficiency in U.S. nursing homes, and wound care ranks among its most common findings. Enhanced Barrier Precautions (EBP) have been surveyed under F880 since April 1, 2024 and are permanently incorporated into SOM Appendix PP (Rev. 229, effective April 2025).*

| Evaluation Criteria                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | F-Tag              | Method  | Score (0–5) | Auditor Notes / Evidence |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|---------|-------------|--------------------------|
| <b>EBP Applied to Every Chronic Wound &amp; Indwelling Device:</b> Enhanced Barrier Precautions are used for ALL residents with a chronic wound (pressure ulcer any stage, diabetic foot ulcer, unhealed surgical wound, venous stasis ulcer) OR an indwelling device (central line, urinary catheter, feeding tube, tracheostomy) — regardless of MDRO status. Gown + gloves are used for high-contact wound care (not merely to enter the room). A peripheral IV does not trigger EBP; a PICC line does. | F880               | O, R, I |             |                          |
| <b>Correct EBP vs. Contact Precautions Triage:</b> Staff correctly apply the drainage distinction: drainage CONTAINED by an intact dressing = EBP (resident not room-restricted, may join group activities); drainage that CANNOT be covered/contained = Contact Precautions (gown + gloves before room entry). Contact Precautions are discontinued as soon as no longer needed to avoid involuntary seclusion.                                                                                           | F880 / F603        | O, I    |             |                          |
| <b>PPE Sequence &amp; Hand Hygiene:</b> Correct donning (Hand hygiene → Gown → Gloves) and doffing (Gloves → Gown → Hand hygiene). Hand hygiene is performed before supplies, before donning, and after doffing; gloves are changed AND hands cleaned when moving from a contaminated to a clean body site.                                                                                                                                                                                                | F880               | O       |             |                          |
| <b>Aseptic Technique (ANTT) &amp; Wound Covering:</b> Clean gloves never contact the wound bed; all “key parts” are identified and protected. All chronic wounds are covered at all times — an uncovered wound is both a healing failure and an infection-control breach.                                                                                                                                                                                                                                  | F880 / F686        | O       |             |                          |
| <b>Supply, Environment &amp; Waste Management:</b> All supplies gathered before entry; single-use items never reused; supply/dressing carts are NOT brought into the resident’s room. Surfaces disinfected with an EPA-registered product immediately after care; soiled dressings discarded in the in-room waste container.                                                                                                                                                                               | F880               | O, R    |             |                          |
| <b>Competency, Escalation &amp; Stewardship:</b> Return-demonstration competency is verified before unsupervised care and at least annually, with documentation. Staff recognize and escalate local (peri-wound erythema, purulent/malodorous exudate), systemic (fever, AMS, hypotension), and spreading/emergency (cellulitis beyond margins, streaking, crepitus — call 911) signs. Antibiotic stewardship: colonization ≠ infection; culture before empiric therapy, then narrow.                      | F880 / F684 / F881 | R, I, O |             |                          |
| SECTION 5 TOTAL SCORE:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |                    |         | ___ / 30    |                          |

## Final Scoring & Performance Category

Carry each section total here. Five sections × 30 = 150 possible points.

| Section                                   | Score            | % of Section |
|-------------------------------------------|------------------|--------------|
| Section 1: Patient-Centered Care          | ___ / 30         | ___ %        |
| Section 2: Multidisciplinary Team         | ___ / 30         | ___ %        |
| Section 3: Interventions & Products       | ___ / 30         | ___ %        |
| Section 4: Process & Protocols            | ___ / 30         | ___ %        |
| Section 5: Infection Control & EBP (F880) | ___ / 30         | ___ %        |
| <b>AGGREGATED TOTAL</b>                   | <b>___ / 150</b> | <b>___ %</b> |
| <b>OVERALL PERCENTAGE</b>                 | <b>___ %</b>     | <b>___ %</b> |

### Outcome Key

A guide only. Individual findings — even in a Center of Excellence — should still be discussed to determine whether a mitigation plan is needed. Any single criterion scored 0–2, or any Section 5 (F880) gap, warrants priority review regardless of the overall percentage.

|                |                                                                                                 |
|----------------|-------------------------------------------------------------------------------------------------|
| <b>90–100%</b> | Center of Skin Health Excellence — aligns with patient-centered outcomes.                       |
| <b>80–89%</b>  | High-Functioning Center — solid program with minor consistency gaps.                            |
| <b>70–79%</b>  | Below Average — requires immediate focus on high-risk (0–2) areas.                              |
| <b>60–69%</b>  | Poor — significant support and process change required to mitigate liability and clinical risk. |
| <b>≤ 59%</b>   | Critical Risk — recommended for external consultation and significant process-flow overhaul.    |

## Action Plan — SMART Goal Development

Review all findings, summarize high-risk items, and discuss action-planning needs with the QAPI team. Prioritize any 0–2 scores and all Section 5 (F880 / EBP) gaps.

### What makes a goal SMART?

|          |                   |                                                                |
|----------|-------------------|----------------------------------------------------------------|
| <b>S</b> | <b>Specific</b>   | Define the goal clearly: who, what, where, and why.            |
| <b>M</b> | <b>Measurable</b> | Determine how you will track progress and know when it's done. |
| <b>A</b> | <b>Achievable</b> | Ensure the goal is realistic given current resources.          |
| <b>R</b> | <b>Relevant</b>   | Align with broader values and long-term objectives.            |
| <b>T</b> | <b>Time-bound</b> | Set a specific deadline to create urgency and focus.           |

### Example — from vague to SMART

|                       |                                                                                                                                                                                                                                   |
|-----------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Before (vague)</b> | <i>"I want to improve our team's communication and off-loading of heels."</i>                                                                                                                                                     |
| <b>After (SMART)</b>  | "We will implement a weekly 15-minute interdisciplinary stand-up, beginning Monday [date], to raise documented staff adherence to heel off-loading from 60% to $\geq 85\%$ within 30 days, verified by the monthly sample audit." |

### Example — Infection Control / EBP (F880)

|                     |                                                                                                                                                                                                                                                                                                          |
|---------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>SMART (F880)</b> | "By [date, within 60 days], 100% of licensed nurses providing wound care will complete an Enhanced Barrier Precautions return-demonstration (correct don/doff sequence and gown+gloves for every chronic wound), raising observed EBP compliance on the monthly audit from [baseline]% to $\geq 90\%$ ." |
|---------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

### Identified Gaps & Possible Themes

**Priority Intervention Selection** (select the most time-sensitive gaps; create as many SMART goals as needed to achieve the overall project goal):

**Project:** \_\_\_\_\_

**Overarching Project Goal:** \_\_\_\_\_



## Build Your SMART Goals

Create individual SMART goals to address each area needed to achieve the overarching project goal. Use one block per goal (at least 3).

| SMART Goal #1                       |                          |
|-------------------------------------|--------------------------|
| Goal statement (S-M-A-R-T):         |                          |
| Linked finding / F-Tag              |                          |
| Stakeholders & assigned champion(s) |                          |
| Budget needs                        |                          |
| Possible risks with the plan        |                          |
| Process / policy changes            |                          |
| Start date                          | _____ Review date: _____ |
| Communication & education plan      |                          |
| Measure of success / how verified   |                          |
| SMART Goal #2                       |                          |
| Goal statement (S-M-A-R-T):         |                          |
| Linked finding / F-Tag              |                          |
| Stakeholders & assigned champion(s) |                          |
| Budget needs                        |                          |
| Possible risks with the plan        |                          |
| Process / policy changes            |                          |
| Start date                          | _____ Review date: _____ |
| Communication & education plan      |                          |
| Measure of success / how verified   |                          |

| SMART Goal #3                       |                          |
|-------------------------------------|--------------------------|
| Goal statement (S-M-A-R-T):         |                          |
| Linked finding / F-Tag              |                          |
| Stakeholders & assigned champion(s) |                          |
| Budget needs                        |                          |
| Possible risks with the plan        |                          |
| Process / policy changes            |                          |
| Start date                          | _____ Review date: _____ |
| Communication & education plan      |                          |
| Measure of success / how verified   |                          |

Follow-Up & Re-Audit Tracker

Schedule and document each review of the action plan. Re-audit the affected criteria, record the new score, and decide to continue, modify, or close.

| Review Date | Goal # | Progress / Findings | Barriers & Adjustments | New Score | Status                    |
|-------------|--------|---------------------|------------------------|-----------|---------------------------|
|             |        |                     |                        |           | Continue / Modify / Close |
|             |        |                     |                        |           | Continue / Modify / Close |
|             |        |                     |                        |           | Continue / Modify / Close |
|             |        |                     |                        |           | Continue / Modify / Close |
|             |        |                     |                        |           | Continue / Modify / Close |

Signatures for Initiation

DON Approval: \_\_\_\_\_ Date: \_\_\_\_\_

Administrator Approval: \_\_\_\_\_ Date: \_\_\_\_\_

Skin Health & Wound Team Lead: \_\_\_\_\_ Date: \_\_\_\_\_